

You selected **3 value drivers**: Patient Throughput (LWBS Reduction), Physician Retention, Level-of-Service Accuracy. Each section walks through the logic step by step—what we're measuring, why it matters, and exactly how we calculated it.

HOURS RETURNED
Documentation time saved

\$72K

Revenue 28%

Assumes 10% annual value growth with increased adoption

Page 1 of 6

Patient Throughput (LWBS Reduction)

\$79K

THE THEORY

When patients leave without being seen (LWBS), the ED loses revenue and—more importantly—fails its core mission. LWBS happens for many reasons: long wait times, staffing constraints, perceived non-urgency. But documentation delays play a role too.

When physicians can complete documentation faster, disposition happens sooner. Patients move through the department more quickly. Wait times compress. And fewer patients give up and leave.

We model this conservatively: only 40% of LWBS is attributable to documentation-driven bottlenecks, and Abridge can address about half of that. The rest is driven by factors outside documentation—true capacity constraints, triage decisions, patient behavior.

YOUR CALCULATION

STEP 1: LWBS PATIENTS

How many patients leave without being seen?

29,250

ED visits x

3.5%

LWBS rate =

1,024 patients leaving

Industry LWBS rates vary from 2-5%. High-volume urban EDs often run higher.

STEP 2: PATIENTS RETAINED

How many can be retained with faster throughput?

1,024

LWBS x

10%

improvement =

102 retained

With faster documentation, staff can see waiting patients before they leave.

STEP 3: ABRIDGE ATTRIBUTION

How many are attributable to Abridge?

102

retained x

33%

attribution =

34 attributed

Portion of improvement directly tied to Abridge documentation efficiency.

STEP 4: REVENUE IMPACT

What's the financial impact?

34

patients x

\$600

per visit =

\$79K

Includes 12% admission rate at \$15K per admission.

BENCHMARK: ED REVENUE PER VISIT

Low Acuity (ESI 4-5)	\$150 - \$250
Medium Acuity (ESI 3)	\$300 - \$450
High Acuity (ESI 1-2)	\$600 - \$1,200+
Blended Average	~\$350

Your input reflects a blended average across your acuity mix.

Annual Patient Throughput (LWBS Reduction) Value

34 recovered x \$600/visit

\$79K

WHAT THIS MEANS

At \$79K, throughput improvements contribute to your total value. This represents approximately 225 patients who would have left but now complete their visit. Beyond the revenue, there's a quality dimension: patients who leave without being seen may delay necessary care or present later with worse outcomes. Reducing LWBS is both a financial and clinical win.

Physician Retention

\$105K

THE THEORY

ED physicians face unique burnout pressures: shift work, high acuity, unpredictable volume, and—yes—documentation burden. The charting follows them home. The cognitive load compounds across shifts.

Replacing an ED physician is expensive: \$500K+ when you factor recruiting, credentialing, lost revenue during vacancy, and the 6-12 months it takes to reach full productivity in a new environment.

We use the same "Rule of Thirds" as outpatient: documentation burden contributes to roughly half of burnout, Abridge reduces that burden by ~70%, yielding ~35% theoretical impact. We round to 30% for conservatism.

Small Provider Count

With fewer than 50 ED providers, retention math is probabilistic over multi-year periods. Over a 3-4 year period, the cumulative effect is equivalent to retaining one additional physician. The value is real—it just materializes over time.

YOUR CALCULATION

STEP 1: EXPECTED TURNOVER

How many departures occur annually?

25

ED providers

x

12%

=

3.0 departures

ED turnover often runs higher than outpatient—8-12% is common.

STEP 2: BURNOUT-RELATED

How many are tied to burnout?

3.0

departures

x

50%

=

1.50 burnout-related

Burnout is particularly prevalent in emergency medicine.

STEP 3: ABRIDGE IMPACT

How many can Abridge help prevent?

1.50

at-risk

x

10%

=

0.15 prevented

The Rule of Thirds: 50% of burnout x 70% doc reduction = 35%, rounded to 30%.

STEP 4: VALUE

What's the savings?

0.15

prevented

x

\$700K

=

\$105K

ED replacement cost is typically higher than outpatient due to specialized credentialing and shift coverage complexity.

BENCHMARK: ED PHYSICIAN REPLACEMENT COST

Recruiting & Credentialing	\$50K - \$100K
Lost Revenue (vacancy)	\$200K - \$400K
Onboarding & Ramp-up	\$100K - \$150K
Total Cost	\$400K - \$650K+

ED replacement typically higher than outpatient due to shift coverage complexity.

Annual Physician Retention Value

0.15 avoided x \$700K

\$105K

WHAT THIS MEANS

At \$105K, retention contributes to your total value. This represents 0.21 avoided departures annually—or roughly one retained physician every 4.8 years. Note: this is a long-term metric—benefits materialize over 12+ months as burnout reduction translates to retention.

Level-of-Service Accuracy

\$72K

THE THEORY

ED visits are often complex—multiple problems, rapid assessment, time-sensitive decisions. But documentation doesn't always reflect that complexity. Under time pressure, notes get truncated. MDM is summarized rather than detailed. The result: visits that should code at 99285 get billed as 99284.

Abridge captures the clinical narrative as it happens. When the note includes the full reasoning—why you ordered that CT, what differential you were considering, how the patient's presentation evolved—coding can be more accurate.

We model a 5% wRVU improvement, consistent with what we see across ED deployments. Your results depend on current documentation quality and specialty complexity.

YOUR CALCULATION

STEP 1: BASELINE WRVUS

What's your current wRVU generation?

29,250

visits

x

2.5

=

73,125 wRVUs

STEP 2: WRVU IMPROVEMENT

How much lift does better documentation create?

73,125

wRVUs

x

3%

=

2,194 wRVU gain

5% improvement assumes meaningful under-documentation today. If your notes are already comprehensive, improvement may be less.

STEP 3: REVENUE

What's the dollar value?

2,194

wRVUs

x

\$33

=

\$72K

Medicare conversion factor. Commercial rates run \$45-65.

BENCHMARK: ED WRVU PER ENCOUNTER

Low Acuity (ESI 4-5)	1.2 - 1.6
Medium Acuity (ESI 3)	2.0 - 2.5
High Acuity (ESI 1-2)	3.0 - 5.0+
Blended Average	~2.2

ED wRVUs vary by acuity mix. 2.0-2.5 per encounter is typical.

Annual Level-of-Service Accuracy Value

2,194 wRVU gain x \$33

\$72K

WHAT THIS MEANS

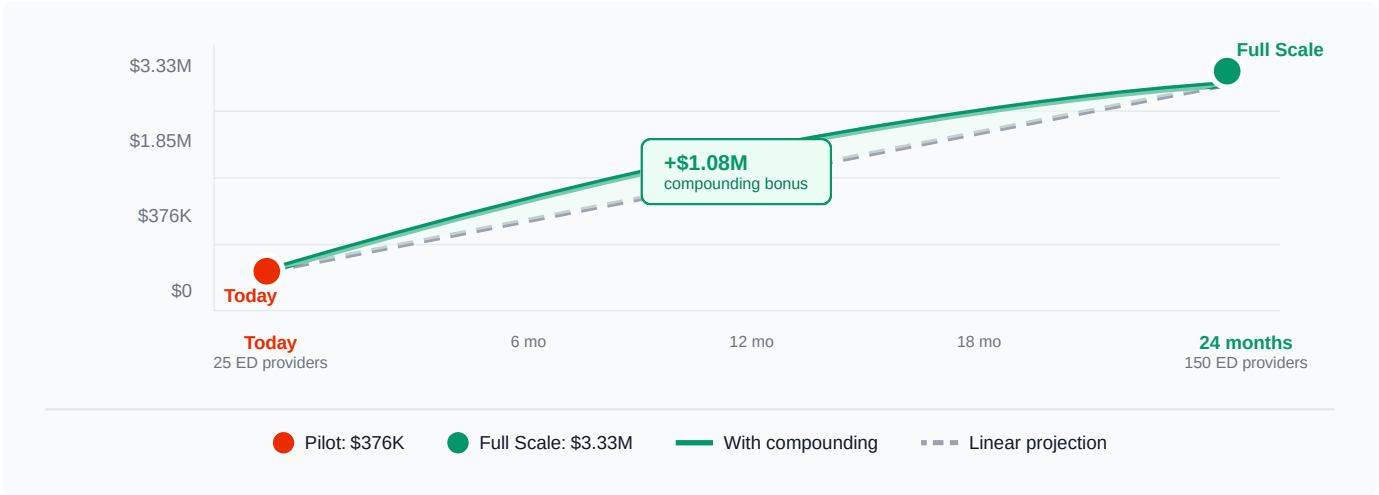
At \$72K, level-of-service accuracy contributes to your total value. This represents getting appropriate credit for the complexity of care you're already delivering. The improvement comes from capturing the full clinical narrative—not upcoding, just accurate documentation of what you're already doing.

Your Journey with Abridge

Start with a pilot. Prove the value. Scale across your organization.

The ROI you see today reflects a pilot deployment of 25 ED providers at 65% utilization. As you scale, value compounds—not just linearly with ED provider count, but exponentially as utilization matures, workflows optimize, and network effects emerge.

At full scale (150 ED providers, 80% utilization), annual value reaches **\$3.33M**—a **8.9x increase** from pilot.



MODEL YOUR SCENARIO

YOUR STARTING POINT

- 25 ED providers
- 45,000 patient visits
- 65% utilization
- \$376K /year

YOUR FULL SCALE POTENTIAL

- 150 ED providers
- 80% utilization
- Scaling: 24 months
- \$3.33M /year

WHY VALUE COMPOUNDS

8.9x

Value doesn't just scale linearly with ED provider count. As utilization improves (65% to 80%), workflows optimize, and network effects emerge, each ED provider generates more value at maturity than at pilot.

DOWNSTREAM VALUE

The ED admission note is just the beginning

When an ED physician decides to admit a patient, their documentation becomes the foundation for inpatient revenue. The conditions they capture, the medical necessity they establish, and the clinical reasoning they document all determine what happens downstream.

Better ED documentation directly impacts:

DRG & CMI Capture

CCs and MCCs documented in ED carry forward to inpatient coding. What's captured here determines your case mix.

Medical Necessity

The admission decision is documented in ED. This is your first line of defense against status denials and downgrades.

CDI Efficiency

When the ED note is complete, CDI teams spend less time querying physicians and more time on complex cases.

Denial Prevention

Payer audits start with the admission note. Complete documentation from day one means stronger appeals.

These benefits are quantified in the **Inpatient Setting**.
If your organization admits patients from the ED, the value compounds when both settings use Abridge.

Methodology & Assumptions

YOUR INPUTS

25 ED providers
45,000 annual patient visits
65% utilization rate
2.5 min saved per encounter
\$200/ED provider/month

ED-SPECIFIC BENCHMARKS

wRVU per encounter: 2.0-2.5
ED physician replacement: \$500K+
Average claim value: \$350
ED turnover rate: 8-12%
ED denial rate: 8-12%

CALCULATION PRINCIPLES

Conservative estimates throughout
Medicare rates (not commercial)
Transparent, auditable logic
No compounding in Year 1
10% annual growth for Y2-Y3

All benchmarks based on aggregate data from 200+ health system partners. Individual results vary based on patient acuity mix, payer mix, operational factors, and implementation quality. We use conservative assumptions throughout—actual value may be higher.

"Emergency departments face unique documentation pressures—high volume, high acuity, time pressure. The difference between a 2x ROI and a 5x ROI usually isn't the technology. It's utilization, change management, and knowing which drivers matter most for your ED's situation."